

FOOD IS MEDICINE INSTITUTE · TUFTS UNIVERSITY · WEBSITE REDESIGN

The homepage, decoded: every decision, toward the brief.

A section-by-section rationale for the pitch mockup, how each screen, image, colour and interaction is a deliberate answer to FIMI's brief, its two decision-maker audiences, and the peer-reviewed evidence. Not aesthetics. An argument.

The three signature numbers this design is built to carry

\$24B

saved a year if every eligible patient received medically tailored meals

20%

fewer hospitalizations after six months of medically tailored meals

121,000

cardiovascular events prevented over 10 years via produce prescriptions

THE NORTH STAR

One job: make a skeptical, time-poor decision-maker believe it in the first screen.

The current site fails because it “presumes people already know who FIMI is.” This redesign is the front door to the food-is-medicine movement for the two people who can scale it nationally, a health-system executive and a policymaker.

POSITIONING, LOCKED INTO THE DESIGN

“The rigorous, nonpartisan academic engine that turns **600+ studies** into **\$24 billion** in annual savings, **20% fewer hospitalizations**, and a pilot a health system can launch this year, or a brief a legislator can cite next week.”

PROVEN, NOT PROMISING

Lead with outcomes that channel desire, then the rigour that kills the skepticism.

CFO-GRADE, NOT WELLNESS

It must feel like clinical & health-economics infrastructure, hard medicine, not a wellness campaign.

THE COMPETITOR IS INDECISION

Every screen ends in one measurable next action.



WHO WE DESIGNED FOR

Two co-primary audiences. Academics, deliberately de-prioritised.

The RFP is explicit: “every page, every design decision... evaluated through their eyes first.”
Every choice in this mockup was made for one of these two people.

PRIMARY 1 • HEALTHCARE LEADER

Health-system exec, hospital & clinical leadership, health-plan leadership.

Their real fear: “‘Food as medicine’ sounds soft, I’ll look naïve betting my P&L on it.”

What they must see: it’s clinically & financially proven, credible peers already do it, and the first step is small and de-risked.

Their move →

MODEL THE SAVINGS FOR YOUR SYSTEM

PRIMARY 2 • POLICYMAKER

Federal & state legislators, agency staff, policy advisors, congressional staff.

Their real need: rigorous, citable evidence they can hand their boss *this week*, a rare bipartisan health win.

What they must see: trusted academic authority, evidence applicable to real decisions, and policy-ready briefs.

Their move →

REQUEST A POLICY BRIEFING

The everyone-else path

Not-yet-ready visitors get one low-commitment step, a prominent Mailchimp newsletter signup near the foot of the page. No dead ends.



THE THREE PRINCIPLES THAT DECIDED EVERY CHOICE

When a design question came up, these broke the tie.

01

Outcome first, mechanism second.

Lead with the number, it channels the desire. Then show the rigour behind it, it kills the skepticism. Never a claim without its citation.

02

Every page is a front door.

Most visitors arrive on a deep page from search. So the signature proof and a clear action live on every template, the numbers repeat, they don't appear once.

03

Production value is the message.

A Rockefeller-grade, expensive-looking site is itself proof of institutional seriousness. Restraint, real data-viz and polish persuade where words cannot.

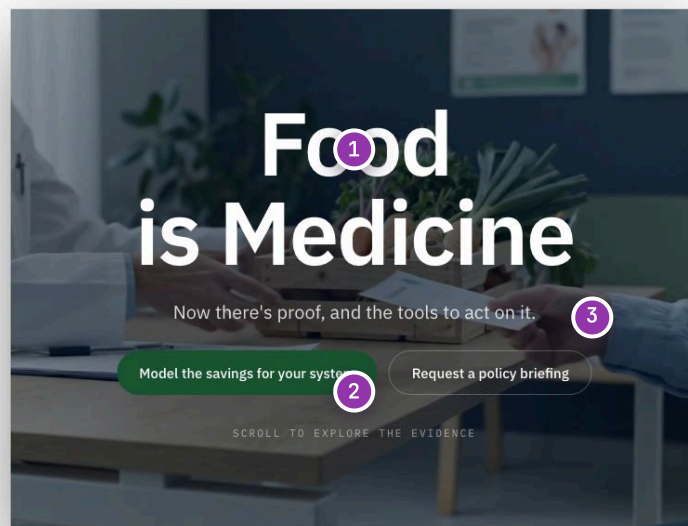
THE USER FLOW

One scroll, engineered as a persuasion sequence.

Desire → proof → relevance → action, for both audiences at once. Every stop hands the reader a reason to keep going and a way to act now.

- 1 Hero, the promise, over real clinical footage** **BOTH**
Food is medicine. Now there's proof, and the tools to act on it. Two audience-split doors.
- 2 The proof, quantified** **BOTH**
The signature numbers, each bound to a peer-reviewed source. This is where skepticism dies.
- 3 The reach, modeled in every state** **POLICY**
An animated US map: "what would this save in *my* district?" The legislator's question, answered.
- 4 Two ways to act, the fork** **BOTH**
Health leaders vs policymakers, each with their value, a proof point, and a low-friction CTA.
- 5 It's already working** **HEALTH**
Real programs, partners, and one human story, kills "nobody actually does this."
- 6 How we work, six focus areas** **BOTH**
The institute's full range, as parallel categories, not a feature dump.
- 7 The close, one clear action** **BOTH**
Implementation footage + the two moves + the authority anchor. The page ends in a decision.

The first screen leads with the promise, over credible medicine, not a stock bowl.



1 The promise, as an H1

The core message leads the first screen, a real H1, not a slide. **\$13**

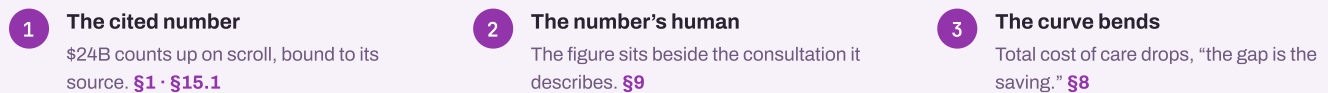
2 Two audience doors

Health & policy CTAs, each low-friction, never “Contact us.” **\$3**

3 Real clinic footage

People, clinics, programs, not a stock food bowl. **\$9**

Better than today, replaces a four-slide “Explore Our Work” carousel that presumed you already knew FIMI. The single biggest change from the current site.



The credibility spine

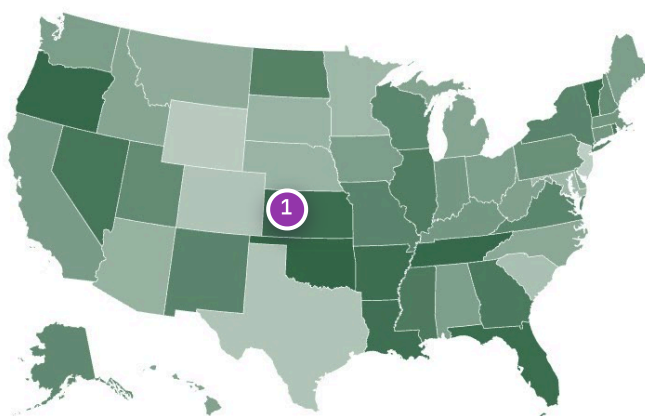
The data-viz that leapfrogs every peer.

02 — THE REACH

Modeled in every state.

FIMI's state-level estimates show what medically tailored meals and produce prescriptions would save in all fifty states. The evidence a legislator needs for their own district.

MODELED SAVINGS **3** HIGH



FIND YOUR STATE

Hover and **2** to see its modeled annual savings.

State-level estimates from FIMI's model, ready for any district.

1 Every state, shaded

A real geographic choropleth on FIMI's 50-state model. **\$8**

2 Your district's number

Hover any state for its modeled annual savings. **\$8**

3 Restrained scale

Low→high, CFO-grade, auto-animating on scroll. **\$15.7**

Better than today, FIMI's 50-state estimates, today a dead PDF list, become a living map a legislator can act on.

The fork, a CFO and a Senate staffer need different things.

ACT ON IT

Two audiences. Two ways to act.

A hospital CFO and a Senate staffer need different things. Pick your door, each one hands you something you can use this week.



FOR HEALTH SYSTEMS & PLANS

1

Model the savings for your population.

- ✓ Net cost impact for your own member mix
- ✓ Benchmarked against peer systems
- ✓ Board-ready from a single pilot

Model the savings Download the toolkit



FOR POLICYMAKERS & AGENCIES

2

Get the evidence for your bill.

- ✓ State-level estimates for any district
- ✓ Peer-reviewed citations ready to quote
- ✓ Nonpartisan, cited by both sides

Request a briefing Browse policy briefs



3

THE TOOLKIT

Ready to use today.

- 1 **Health-systems door**

Model the savings, board-ready from one pilot. **\$2**
- 2 **Policymakers door**

Citable, state-level evidence for a bill. **\$2**
- 3 **One real proof point**

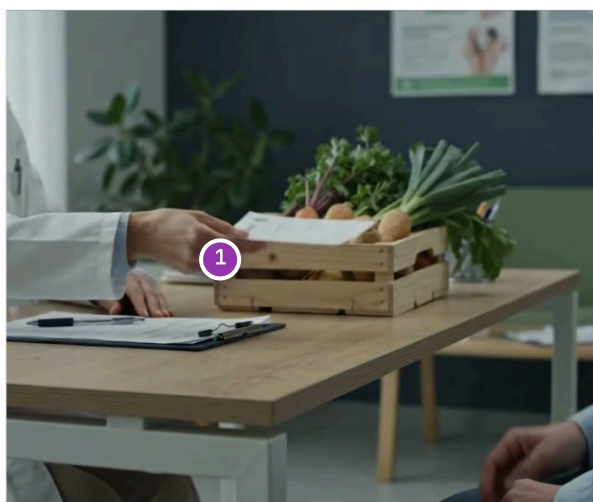
A Massachusetts Medicaid case, in meal-tray footage. **\$2**

Better than today, replaces org-chart navigation with two audience-first paths, each usable this week.

Feasibility, told through a human, with an honest label.

It's already working.

Not a theory. Real health systems, payers, and states are running food is medicine today, with FIMI's evidence and tools behind them.



A STORY FROM THE FIELD

"My doctor didn't just tell me to eat better. She wrote me a prescription for food. Six months on, my blood sugar is down and I haven't been back to the ER."

Maria R.
Enrolled in a medically tailored meals program

16% **Fewer**
low cost of care emergency visits

REPRESENTATIVE STORY · ILLUSTRATIVE FOR THIS PITCH

VOICES FROM THE FIELD

PARTNERS & PROGRAMS

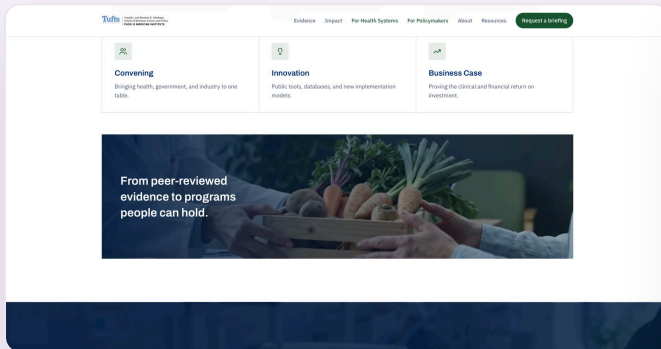
Running today, across the country.

- 1 A human, not a logo**
A real face and a food-prescription moment. **\$3**
- 2 The outcome, measured**
16% lower cost of care; fewer ER visits. **\$2.2**
- 3 Labelled representative**
Honest for a pitch, not overclaimed. **\$3**

Better than today, replaces zero social proof with tangible, human feasibility.

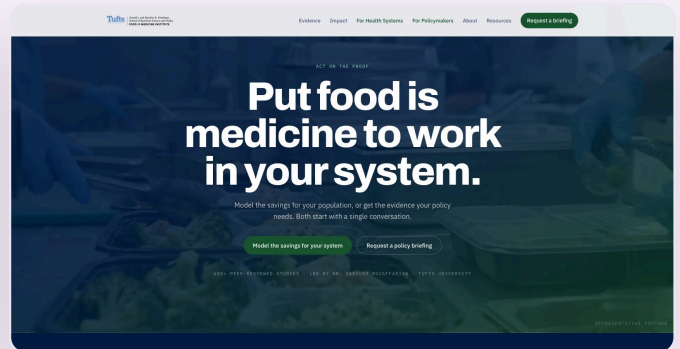
SECTION RATIONALE · HOW WE WORK & THE CLOSE

The full range, then a decisive ending.



SIX FOCUS AREAS, AS PARALLEL CATEGORIES

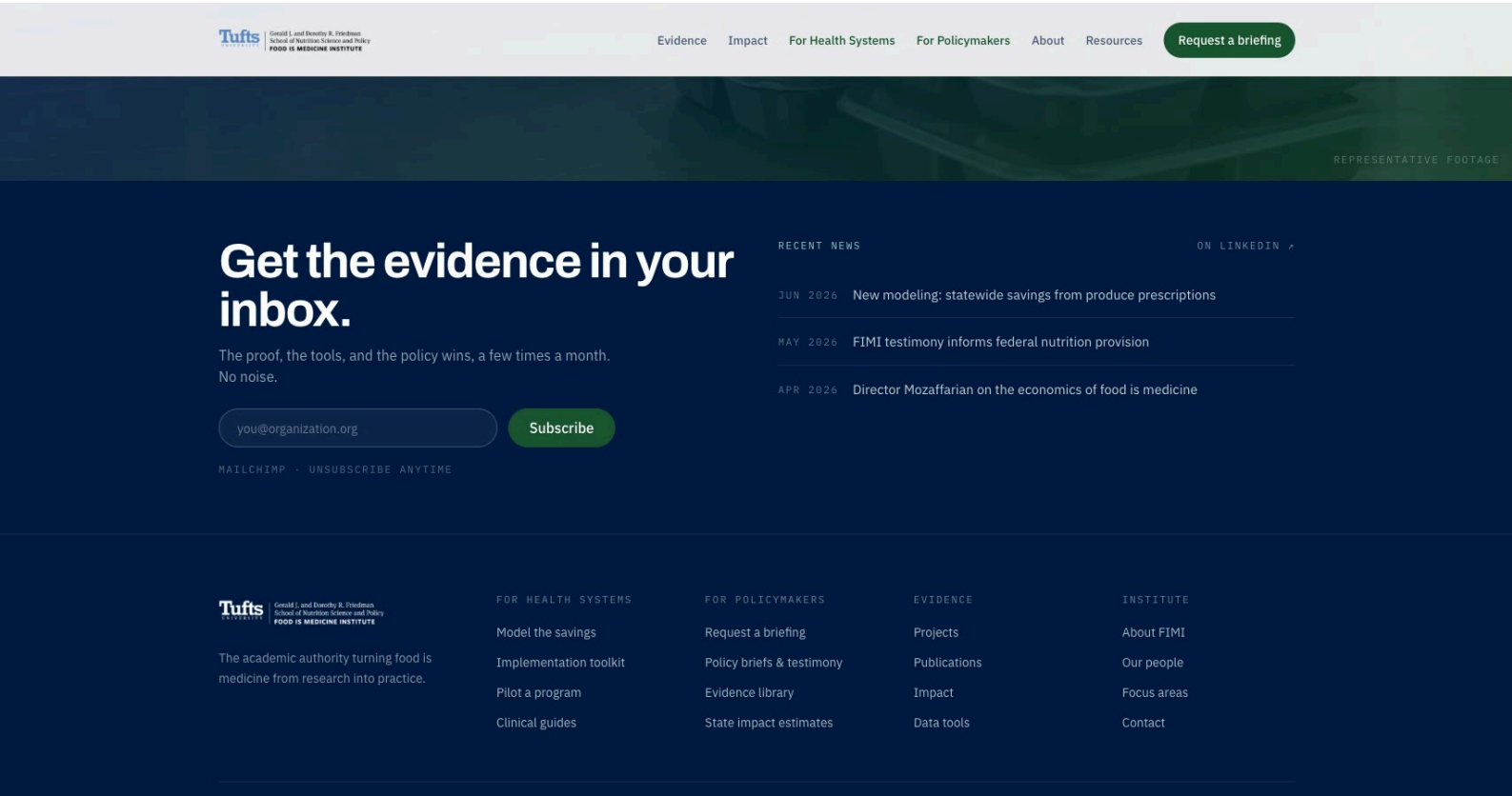
Evidence · Practice · Policy · Convening · Innovation · Business Case, each a distinct mark, not a false 01 to 06 sequence, closed by a photographic band. **Categorical, not decorative numbering.**



THE CLOSE, IMPLEMENTATION FOOTAGE + AUTHORITY

Meals coming off the line, the two actions, and the authority anchor: **600+ studies · Dr. Dariush Mozaffarian · Tufts. Every screen ends in a measurable action.**

Where today’s site has zero links, this closes every loop.



REQUIRED MODULES, ALL PRESENT

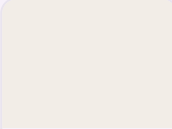
Mailchimp signup · LinkedIn · date-stamped news · audience wayfinding · Tufts + FIMI lockup · Privacy · **Accessibility (WCAG 2.1 AA)** · Cookie settings. **RFP \$4.4.**

ACCESSIBILITY, BAKED IN

Meaningful alt text on every image · text contrast to AAA on navy · reduced-motion honoured · visible focus states · 44px tap targets. **The current site fails all of these.**

Not “Tufts-ish.” Verified against brand.tufts.edu.

These are FIMI's own Tufts colours, checked against the exact source the RFP §4.3 cites. **Six core tokens are exact official Tufts matches:** a legitimate, distinct FIMI application (navy + green for CFO authority + health, in place of the university's blue + brown). (This document, by contrast, wears Zyware's brand.)

						
Jumbo Blue #002E6D ✓	Tufts Blue #3E8EDE ✓	Prez Lawn Green #1A5532 ✓	Aurora Green #98D5C7 ✓	Candle White #F2EDE7 ✓	Beacon Yellow #FFC72C ✓	Alert Red #C0311A ✓

TEXT · AUTHORITY

Jumbo Blue on white $\approx$ **12:1** (AAA). The serious, CFO-grade base.

ACTION · PRIMARY BUTTONS

White on Prez Lawn Green $\approx$ **8:1**, fixes the old site's failing green (3.0:1).

ACCENTS · NEVER SMALL TEXT

Tufts Blue reserved for large fills & borders, per Tufts' own text rule.

THE SYSTEM • TYPE, MOTION, DATA & IMAGERY

Four quiet systems doing the persuading.

TYPOGRAPHY

An editorial display voice for authority, a legible humanist sans for evidence copy, and a **monospace for data & citations**, the clinical-readout voice that reads as hard medicine, not wellness. One tight scale replaces the current site's 12 unrelated sizes.

MOTION

Purposeful and restrained. Counters tick up on scroll; sections reveal once, single-direction; no parallax pile-ups. **Reduced-motion is honoured**, final states shown with no count. Motion signals production value, never gimmick.

DATA VISUALISATION

Dynamic but non-interactive, exactly as FIMI asked: **animated counters, a live US map, a cost-of-care trend**. Precise, restrained, CFO-grade, the piece that makes it feel like economics, not a blog.

IMAGERY

People, clinics and real programs, a clinician's consult, a food prescription, a medically-tailored-meal line. One visual per section. Representative footage in this mockup; swaps to FIMI's library + the Oct 2026 brand video.



Why it's this clean: we removed everything that wasn't proof.

A calm, disciplined page reads as a serious institution; a busy, colourful one reads as a wellness blog. The restraint isn't a look, it's the credibility strategy the brief asks for: *"restraint, real data-viz and polish do the persuading that words cannot."* Nothing here is decorative. Every element earns its place against the proof, or it is cut.

SPACE, NOT DIVIDERS

Hierarchy comes from generous whitespace and one tight type scale, not decorative lines, heavy boxes, or chrome. Editorial confidence, not dashboard clutter. §9: **today's site is "dense and flat"; this is editorial and confident.**

COLOUR SIGNALS, NEVER DECORATES

Navy = authority · green = action & health · warm neutral = relief. No rainbow. Colour that carries meaning reads as hard economics; colour used for fun reads as wellness, and a disciplined palette can never be read as partisan.

WHITE-LED, NAVY-ANCHORED

A modern, calm canvas with navy moments placed for weight. It sidesteps both the warm-cream "AI-default" trap and any partisan-coded palette. §15.8: **navy-led, not cream-led.**

RESTRAINT = PRODUCTION VALUE

An expensive-looking calm is itself proof of institutional seriousness, the costly signal a CFO and a legislator both read instantly. **Production value is the message.**

THE CLIENT'S WHOLE PALETTE, IN PLAY AT ONCE, THREE TONES CARRY THE PAGE; TWO ACCENTS APPEAR ONLY IN FLASHES

NAVY · AUTHORITY

GREEN · ACTION

CANDLE · RELIEF

WHAT WE DELIBERATELY CHANGED

Every “don’t” from the audit, answered.

THE CURRENT SITE

- Four-slide “Explore Our Work” hero carousel
- Stock / generic imagery, 11 of 11 with no alt text
- Academic “Aim One / Two / Three” framing
- Dead links; a footer with zero links
- Unlabelled, uncited statistics
- Bright green button at 3.0:1 contrast (fails)
- No H1s; no clear audience entry points

THIS MOCKUP

- Single proof-led hero, one promise, two doors
- Real clinic / program footage, alt text on every image
- Outcome-first, plain-language, for a 20-second skim
- Real wayfinding + full compliance footer
- Every number bound to its peer-reviewed source
- Prez Lawn Green #1A5532 at ~8:1 (AA/AAA)
- Audience-first nav: Evidence · Impact · Health · Policy

This homepage is the front door. Here's the rest of the build.

01 • THE TIE-BREAKER

The \$7.3 project page (Delta GREENS), rebuilt outcome-first with live, cross-linked data-viz, the RFP's own test of a true design thought-partner.

02 • HERO, ELEVATED

Lift the animated \$24B · 20% · 121,000 and a drawn evidence curve into the hero, the page's one orchestrated moment.

03 • REAL ASSETS

Swap representative footage for FIMI's photo library and the October 2026 brand video & interviews.

04 • BUILT TO RUN

Mega-menu, and a CMS a non-technical team can update directly, the client's stated primary consideration.